

SHINTAMET®

FILM COATED TABLET 200MG

Cimetidine is the first successful histamine H₂-receptor antagonist. It is indicated in the short-term treatment of active duodenal ulcer. It offers a variety of dosage regimens that are very effective in healing duodenal ulcers and in providing rapid pain relief.

Ingredient(s):

Each tablet contains:

Cimetidine 200mg

Pharmacology (Summary of Pharmacodynamic and Pharmacokinetic):

1. Cimetidine competitively inhibits the action of histamine at the histamine H₂-receptors of the parietal cells.
2. Cimetidine, although not an anticholinergic agent, has been shown to inhibit both daytime and nocturnal basic gastric acid secretion stimulated by food, histamine, pentagastrin, caffeine and insulin.
3. Cimetidine reduces total pepsin output as a result of the decrease in volume of gastric juice. Gastric pH is increased to 5.0 or greater.
4. Cimetidine has no effect on lower esophageal sphincter (LES) pressure or the rate of gastric emptying.
5. Cimetidine is rapidly absorbed after oral administration and peak levels occur in 45 ~ 90 minutes. Half-life is approximately 2 hours.
6. The majority of the drug is recovered in the urine as the unchanged compounds, with smaller amounts of metabolites. Up to 10% of the dose is eliminated in the feces.

Indication(s):

Treatment of duodenal and gastric ulcer, esophageal reflux disease (including heartburn and peptic esophagitis), selected cases of persistent dyspepsia, and pathological hypersecretory states, such as the Zollinger-Ellison Syndrome.

Dosage and Administration:

The total daily dose by any route should not normally exceed 2.4g.

1. Duodenal ulcers:
400mg two times a day, in the morning and at bedtime; 800mg at bedtime; or 200mg three times a day, with meals and 400mg at bedtime (total dose is 1.0g).
The dose can be increased to 400mg four times a day, with meals and at bedtime (total daily dose is 1.6g). Treatment should be continued for at least 4 weeks.
2. Prophylaxis of recurrent duodenal ulcer:
400mg at bedtime, for 6 months.
3. Benign gastric ulcer:
400mg in the morning and at bedtime, or 200mg three times a day with meals and 400mg at bedtime. Treatment should be continued for 6 weeks. For recurrent gastric ulcer, 400mg at bedtime.
4. Reflux esophagitis:
Mild to moderate condition: 400mg two times a day.
More severe conditions: 400mg four times a day with meals and at bedtime for 12 weeks.
5. Zollinger-Ellison Syndrome and other gastric hypersecretory conditions:
The usual dose is 200mg three times a day with meals and 400mg at bedtime. It can be increased to 400mg four times a day. The dosage can be adjusted as needed and therapy continued for as long as clinically indicated.
6. Dyspepsia: 200mg four times a day with meals and at bedtime for 4 weeks.
Diagnosis should be made if symptoms are not improved in 2 weeks.

The dosage of Cimetidine should be reduced in patients with impaired renal function; suggested doses according to creatinine clearance are: creatinine clearance of 0 to 15ml per minute, 200mg two times daily; creatinine clearance of over 50ml per minute, normal dosage.

Children: 1 - 12 years: 20 - 25 mg/kg daily in divided doses.

To be dispensed on physician's prescription.

Route of administration: oral

Contraindication(s):

There are no clear contraindication to drug use other than hypersensitivity to Cimetidine.

Precaution(s) / Warning(s):

1. The possibility of gastric malignancy should be ascertained since the symptoms may respond to Cimetidine treatment without actual healing.
2. Because Cimetidine is excreted by the kidney, a reduced dosage should normally be administered to patients with impaired renal function.
3. Because the formation of the organs in the newborn and neonate are not yet complete, risk should be weighed against benefit before the product is given.
4. Cimetidine should be used in pregnant or lactating patients only when the anticipated benefits outweigh the potential risk.
5. In patients on drug treatment or with illness that could cause falls in blood cell count, the possibility that H₂-receptor antagonist could potentiate this effect should be borne in mind.

Drug Interaction(s):

Cimetidine prolong the elimination of drugs metabolised by oxidation in the liver. Drugs which have the pharmacological interaction with Cimetidine are theophylline, phenytoin, lidocaine, antiarrhythmics, benzodiazepins, some β -blockers. Cimetidine has been reported to reduce the hepatic metabolism of warfarin type anti-coagulant; therefore, close monitoring of prothrombin time is recommended. Simultaneous administration of Cimetidine and antacid is not recommended; since antacids have been reported to interfere with the absorption of Cimetidine.

Side Effect(s) / Adverse Reaction(s):

1. Altered bowel habit, mild and transient diarrhea, headache, tiredness, dizziness, somnolence and rash have been reported.
2. Reversible impotence have been reported in patients with pathological hypersecretory disorders, e.g., Zollinger-Ellison Syndrome.
3. Regularly observed small increases in plasma creatinine and some increases in serum transaminase have been reported.
4. Gynecomastia has been reported in patient treated for one month or longer.
5. Reversible confusional states, e.g., mental confusion, agitation, psychosis, depression, anxiety, hallucinations, disorientation, have been reported predominantly.
6. Reversible liver damage.
7. Rarely, decrease blood count, alopecia, muscle and joint pain, bradycardia and AV blocks, interstitial nephritis and pancreatitis.

Symptoms and Treatment for Overdosage, and Antidote(s):

No significant adverse effects have been reported that associated with gross deliberate overdosing. Since there is no specific antidote for overdose with histamine H₂-receptor antagonists, treatment is symptomatic and supportive with possible utilization of emesis and/or gastric lavage, clinical monitoring, artificial respiration in respiratory failure, and administration of beta-blocker to control tachycardia.

Storage Condition(s):

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

Shelf-Life:

3 years from the date of manufacture.

Product Description and Packing(s):

An orange color film coated tablet.

Plastic bottle of 200's, 500's and 1000's (for export only).

Blister packing: 10's x 10, 10's x 20, 10's x 50, 10's x 100.



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